

Assessment of Respiration

STUDENT'S ACTIVITY

A 68-year-old man was diagnosed with chest infection 10 years ago. He has a 40-year smoking history (is still smoking) and has been hospitalized twice due to chest infections during the last 12 months. He has trouble getting his breath. How do you assess the patient's condition?

ALTERATION IN BREATHING PATTERNS

Bradypnea: The respiratory rate is abnormally slow (less than 12 breaths per minute) Occurs in coma due to cerebral hemorrhage or large doses of sedatives,

Tachypnea: The respiratory rate is abnormally rapid (greater than 20 breaths per minute)

Apnea: Respirations cease for several seconds.

Hyper ventilation: Rate and depth of respirations increase.

Hypoventilation: Rate is abnormally low and depth is shallow. Shallow respiration occurs in diseases of the lung such as pneumonia and pleurisy.

Sighing or air hunger: Indicates a need for more oxygen. Occurs in serve hemorrhage diabetic coma or due to simulation of respiratory center by excess of acid.

Wheezing: Sound made during expiration may be due to obstruction in the lower respiratory tract as in the case of asthma.

Stertorous breathing: Noisy snoring inspiration occurs in unconscious patients which may be due to the tongue slipping back. Peculiar hissing respiration occurs in uremic coma.

Orthopnea: Inability to breath easily unless in an upright position.

Dyspnea: Difficult breathing. If it is during inspiration it is due to laryngeal obstruction; if it is during expiration it is due to Asthma.

Cheyne stokes or periodic breathing: Alternative periods of hyperpnoea, occurring in a rhythmical cycle It is important to note this phenomenon as this is a serious sign.

Asphyxia: Occurs due to lack of oxygen supplied to the cells. This is found in drowning patients of persons who have inhaled poisonous gases.

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